

PRICE DROP

● Monthly semaglutide therapy in India now starts from about ₹1,290 for low-cost vial formulations

MONTHLY COST (₹)	COMPANY	BRAND	FORMAT
₹1,290	Natco Pharma	Semanat / Semafull	Vial
₹1,290	Eris Lifesciences	Sundae	Vial
₹1,300–1,760	Glenmark Pharmaceuticals	GLIPIQ	Vial
~₹1,800+	Alkem Laboratories	Semasize / Obesema	Pen
~₹2,200	Zydus Lifesciences	Semaglyn / Mashema / Alterme	Pen
₹3,000–8,000	Sun Pharmaceutical Industries	Noveltreat / Sematrinity	Pen
₹3,999	Torrent Pharmaceuticals	Semalix / Sembolic	Injectable + Oral
₹4,200	Dr. Reddy's Laboratories	Obeda	Pen
₹4,000–4,500	Natco Pharma	-	Pen
₹5,600–16,000	Novo Nordisk	Ozempic / Wegovy	Pen

SOURCE BT RESEARCH

56 | Western healthcare systems where insurers absorbed much of the cost. In India, where the National Health Accounts data show that out-of-pocket spending accounts for 39% of total health expenditure, those prices created an impregnable barrier for most patients. On the other hand, India’s pharma companies—among them Sun Pharma, Torrent, Cipla and Dr Reddy’s Laboratories alongside dozens of mid-sized players—have built their businesses around reverse-engineering molecules at scale and driving down costs through optimised domestic supply chains. The speed of entry into the semaglutide generic market, with more than 40 players filing within weeks of patent expiry, highlights both the commercial scale of the opportunity and the manufacturing depth the sector has developed here over decades. “We have never seen these many companies move this fast into a

single therapeutic category. The level of competition entering this market will compress prices further over the next 12 to 18 months,” says Rajesh Pherwani, Founder and Chief Investment Officer at Valcreate Investment Managers LLP. However, the limited cold chain infrastructure remains a major logistical hurdle. Semaglutide is a delicate peptide that must be stored between 2°C and 8°C. “Semaglutide is a complex protein molecule in form of a chain of amino acids. We use the advanced r-DNA technology to develop this peptide molecule which requires rigorous control at every stage to ensure product quality, stability and careful management of impurity profiles,” says Vikrant Shrotriya, MD of Novo Nordisk India. Even so, the early pricing trends are striking. Branded semaglutide injections were priced above ₹10,000 a month before generic entry. Many new entrants are target-

ing price points below ₹2,000, with analysts projecting that sustained competition could even push costs below ₹1,500 within 18 months. Clinicians and health economists caution that affordability is not the only factor. A monthly cost of ₹2,000 is accessible for a salaried professional in a major city. “Whether the drug eventually reaches patients in district hospitals or lower-income groups will depend on insurance coverage, government procurement decisions and absorptive capacity of the public health system,” says Pherwani. THE TREATMENT GAP Research by the Indian Heart Association shows that less than 50% of patients here stick to therapies for chronic conditions, including hypertension and type II diabetes. “Patients start treatment, improve and stop. They return when symptoms worsen. The episodic pattern of care is not easily disrupted by just a cheaper drug,” says Dr Misra. Semaglutide presents specific challenges in that context. It is injected into the fatty tissue, requiring familiarity and comfort that oral medications do not. It is intended for long-term use, with clinical trial data published in *The New England Journal of Medicine* showing that benefits reverse when patients discontinue. “The drug works for eligible patients. But it must be accompanied with counselling, follow-up, adherence. Without that, patients stop after some months and it isn’t successful,” says Dr Ambrish Mithal, Chairman and Head of Endocrinology and Diabetes at Max Healthcare. The readiness of physicians is another gap. A 2024 survey by the Research Society for the Study of Diabetes in India found that less than a third of general practitioners in non-metro cities felt confident initiating GLP-1 therapies.